

Healthcare IT Fundamentals – Section 2, Lecture 3

Patient Safety, Clinical Decision Support (CDS) & Medication Error Prevention (Simple Notes)

Big Picture

In previous lectures, we learned:

- EHR architecture
- Clinical workflows
- Data integration
- Structured vs unstructured data

This lecture focuses on:

How EHR systems actively prevent patient harm.

Main topic:

Using technology to reduce medication errors and adverse events.

1. The Problem with Paper-Based Healthcare

Before EHRs, healthcare relied heavily on:

- Paper charts
- Handwritten prescriptions
- Human memory

This created many opportunities for mistakes.

Example Scenario

Patient visits a specialist.

Doctor writes:

Prescribe Drug A

Problems:

- Patient forgets to mention allergy
- Doctor doesn't know all medications
- Pharmacist misreads handwriting

Result:

✗ Wrong medication

✗ Allergic reaction

✗ Serious harm

Why Paper Systems Are Risky

Paper systems depend on:

Doctor Memory

+

Patient Memory

+

Pharmacist Interpretation

Humans make mistakes.

Swiss Cheese Model

A famous patient safety concept.

Imagine multiple slices of Swiss cheese.

Each slice is a safety layer.

Example layers:

1. Patient
 2. Doctor
 3. Pharmacist
 4. Nurse
-

Problem

Every layer has holes.

```
Layer 1:  O
Layer 2:   O
Layer 3:    O
Layer 4:     O
```

If holes line up:

Error reaches patient.

Example

Patient forgets allergy

↓

Doctor misses allergy

↓

Pharmacist misses allergy

↓

Patient receives medication

↓

2. How EHR Improves Patient Safety

Modern EHR acts like an automatic safety inspector.

It continuously checks for risks.

Example

Doctor enters medication through CPOE.

```
Prescribe Penicillin
```

System instantly checks:

- Allergies
 - Existing medications
 - Medical history
 - Dosage rules
-

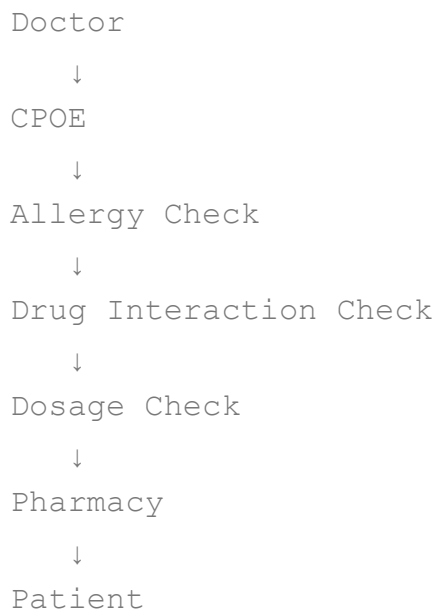
If Problem Found

System generates alert.

```
WARNING:  
Patient allergic to Penicillin
```

Error caught before reaching patient.

Safety Layers in EHR



Multiple automated defenses.

3. Clinical Decision Support (CDS)

CDS is the intelligence layer of EHR.

Purpose:

Help clinicians make safer decisions.

Real-Time Alerts

CDS monitors actions as they occur.

Example 1

Drug Allergy Alert

Patient allergic to Drug X

Example 2

Drug Interaction Alert

Patient already taking:

Blood Thinner

Doctor prescribes:

Drug B

System detects dangerous interaction.

Example 3

Dosage Alert

Doctor enters:

5000 mg

Normal dosage:

500 mg

System warns immediately.

4. Soft Stops vs Hard Stops

Not all alerts are equal.

Soft Stop

Warning appears.

Doctor can continue after providing reason.

Example

```
Warning:
Potential interaction detected.

Continue?
```

Doctor may override.

Hard Stop

System blocks the action.

Cannot continue.

Example

```
Severe contraindication detected.
Order blocked.
```

Doctor must change order.

Easy Memory Trick

Type	Meaning
Soft Stop	Warning
Hard Stop	Block

5. Evidence-Based Medicine

Healthcare should follow proven medical guidelines.

CDS helps enforce these standards.

Problem Without CDS

Doctor must remember:

- Tests
- Medications
- Procedures
- Guidelines

for hundreds of diseases.

Impossible to remember everything.

Solution: Order Sets

Predefined treatment templates.

Example: Pneumonia

System automatically suggests:

- Chest X-ray
- Blood tests
- Antibiotics

based on current clinical guidelines.

Instead of:

`Doctor remembers everything`

System assists.

Benefits

- ✓ Consistent care
 - ✓ Fewer missed steps
 - ✓ Reduced cognitive load
 - ✓ Better patient outcomes
-

6. Standardization of Care

Without EHR:

Different doctors may treat same condition differently.

Example:

Doctor A orders:

5 tests

Doctor B orders:

2 tests

Variation increases risk.

With standardized order sets:

Everyone follows evidence-based workflow.

Result:

More predictable outcomes.

7. Alert Fatigue

One of the biggest challenges in Healthcare IT.

What is Alert Fatigue?

Too many alerts.

Doctors become desensitized.

Example:

Alert
Alert
Alert
Alert
Alert
Alert

Eventually:

Click Ignore

without reading.

Risk

Critical alerts may also be ignored.

Real-World Problem

90% of alerts may be low-value.

Doctors learn to dismiss everything.

Solution

Organizations must:

- Remove unnecessary alerts
 - Prioritize critical warnings
 - Monitor override rates
-

8. Governance Committees

Hospitals cannot randomly modify EHR rules.

Special committees manage:

- CDS rules
 - Alerts
 - Order sets
 - Clinical workflows
-

Responsibilities

Review:

- Alert effectiveness
 - Safety incidents
 - User feedback
-

Goal:

Keep system useful and safe.

9. Downtime Procedures

Technology can fail.

Servers can go down.

Networks can fail.

Power can fail.

Important Question

What happens if EHR is unavailable?

Hospitals need:

Downtime Procedures

Example

During outage:

- Paper forms used
 - Manual documentation used
 - Backup workflows activated
-

Why Important?

Patient care must continue even if systems fail.

10. Role of Administrative Staff

Patient safety is not only the doctor's responsibility.

Everyone contributes.

Administrative staff must:

- Report hardware issues
- Verify patient information
- Follow downtime procedures

- Protect equipment
-

Example

Broken workstation not reported.

Doctor cannot access records.

Patient care delayed.

11. System Uptime

Healthcare systems must be highly available.

Example

Banking downtime:

Annoying

Hospital downtime:

Potentially life-threatening

This is why healthcare systems need:

- Backup servers
 - Disaster recovery
 - Power backups
 - Monitoring
-

12. User Access Tracking

Every user gets unique credentials.

Never share accounts.

Why?

Auditability.

System can track:

Who

Did What

When

Example

Medication changed.

System records:

User: Dr Smith

Time: 10:05 AM

Action: Medication Updated

Critical for:

- HIPAA compliance
 - Investigations
 - Patient safety reviews
-

13. Human Side of Technology

Technology helps.

But technology can also create new problems.

Screen-Staring Problem

Doctor focuses on computer.

Patient feels ignored.

Bad Experience

Doctor → Screen
Patient → Waiting

Patient-provider relationship suffers.

Triangle of Care

Best practice.

Arrange:

Patient
/ \
/ \
Doctor---Computer

All three participate.

Benefits

Patient sees:

- Test results
- Charts
- Images

during discussion.

Technology becomes collaborative.

Voice Recognition

Newer EHRs increasingly support:

- Speech-to-text
 - Voice dictation
 - AI-assisted documentation
-

Benefits:

- ✓ More eye contact
 - ✓ Faster documentation
 - ✓ Less typing
 - ✓ Better patient interaction
-

Key Takeaways

EHR Improves Safety

Through:

- CPOE
 - CDS
 - Allergy checks
 - Interaction checks
 - Dosage validation
-

Soft Stop

Warning only.

Hard Stop

Action blocked.

Order Sets

Standardized treatment templates.

Alert Fatigue

Too many alerts causing users to ignore them.

Governance

Committees manage CDS rules and alerts.

Downtime Procedures

Backup workflows when EHR is unavailable.

Audit Trails

Track who did what and when.

Triangle of Care

Patient + Provider + Computer working together.

End of Section 2 Summary

Section 2 taught us:

EHR Architecture

What components exist.

Clinical Workflow

How data flows through the system.

Patient Safety

How CDS and CPOE reduce errors.

Data Quality

Why structured, accurate documentation matters.

Integration

How departments share information.

Human Factors

Technology should support care, not replace human interaction.

For a Ruby on Rails Developer

Think of CDS as:

```
before_save :validate_patient_safety
```

Before a medication order is committed:

```
check_allergies  
check_drug_interactions  
check_dosage  
check_contraindications
```

If validation fails:

```
raise PatientSafetyError
```

In simple terms:

Clinical Decision Support is like a giant real-time validation layer for healthcare, ensuring dangerous actions are caught before they affect the patient. 🚀